Protecting Providers from Penalties for Documented Patient Refusals in Quality Measures

- CMS should ensure quality measures include **patient refusal exclusions** to avoid penalizing providers unfairly.

- Measures for breast cancer, colon cancer, A1C, and BP control often lack standardized refusal handling across programs (e.g., MIPS vs. Star Ratings).

- Repeated outreach to patients who decline care creates inefficient care loops, **staff burnout**, and **verbal escalation toward nurses and MAs**.

- Providers face **pressure** from **health plans, CDOs,** and HEDIS teams to repeatedly "nag" patients despite refusals without clinical benefit.

- Technology vendors (TD) must support structured refusal documentation (e.g., SNOMED CT codes) to enable fair measure exclusions.

- CMS should address **health IT ecosystem misalignment** — providers (PR) are penalized while patients (PC) can refuse and vendors (TD) may not support refusal capture.

- Remote monitoring data (e.g., BP cuffs, CGMs) should be **integrated and measured appropriately**, with patient opt-outs excluded from penalties.

- CMS must ensure that value-based care models (VB) reward **actual provider effort, not just patient behavior.**

CMS and NCQA should revise quality measurement logic for cancer screenings (e.g., colorectal and breast cancer), diabetic eye exams and immunizations (e.g., pneumococcal) to allow for structured documentation of **patient refusal** as a valid **denominator exclusion**.

List of quality measures where the providers and plans **are penalized with patient refusals**:

- Breast cancer screening (mammogram)
- Colorectal cancer screening (colonoscopy, FIT, FOB)
- Diabetic eye exam
- Osteoporosis screening (bone density) post fracture
- Statin therapy for patients with cardiovascular disease

- Statin use in persons with diabetes
- Depression screening (PHQ-9)
- Tobacco cessation counseling
- ACE-ARB medication adherence 90 day fill
- Statin medication adherence 90 day fill
- Diabetes medication adherence 90 day fill
- Transition of care measures (med rec post discharge, etc.)

Currently, providers and plans are penalized even when patients decline procedures through shared decision-making. This means

- **providers' performance percentages decrease**
- **star ratings decrease**

when patients decline care, **despite shared decision-making**.

Health plans are under significant pressure to meet HEDIS and Star measure thresholds and often push providers to close care gaps regardless of patient preference. This creates systemic pressure on providers to **prioritize metrics over ethics**, particularly for invasive or culturally sensitive screenings.

CMS must revise quality measurement logic to protect **providers from performance penalties** when patients make informed, **documented refusals of recommended services.** Under current rules, providers are held accountable for actions outside their control — including patient refusals of screenings (e.g., mammograms, colonoscopy, cervical cancer), preventive medications (e.g., statins), and behavioral health assessments (e.g., PHQ-9).

CMS should expand the allowance for documented patient refusals to medication-related quality measures such as ACE Inhibitor/ARB therapy for diabetes with hypertension, statin therapy initiation, and adherence measures (e.g., 90-day fills). Penalizing providers for patient medication refusals or non-adherence without proper denominator exclusions creates misaligned incentives and undermines patient-centered care.

Refusal concepts already exist in SNOMED CT for the following:

Advance care planning declined (situation): 713615000
Annual visit declined (situation): 26631000175103
Blood glucose test declined (situation): 698473004
Blood pressure procedure declined (situation): 413123006
Bone density scan declined (situation): 720833006
Colonoscopy declined (situation): 116101000119100
Depression follow-up declined (situation): 9511000175107
Discussion about advance care planning declined (situation): 714747005
Mammogram declined (situation): 720835004

Osteoporosis treatment declined (situation): 116491000119104
Procedure declined (situation): 1296859006

Blood glucose test declined would apply to the Glycemic Status Assessment for Patients with Diabetes (GSD) measure.

Blood pressure procedure declined would apply to the Controlling Blood Pressure (CBP) measure.

Depression follow up declined would apply to the depression remission after 12 months measure.

Quality measure pressure led to **increased** provider, nurse and administrative **burden**:

- Repeat conversations between providers and patients to complete the quality measures by December 31st
- Pressure from health plans, CDOs, HEDIS experts to nag patients (ask patient at every visit or telephone call) regardless of refusals
- Patient refusals triggering to escalating behavior (shouting) towards nurses/MA's
    - Providers having to step in to calm the patients
- Frequent rechecks for BP due to refusal of low salt diet
- Persistent elevated A1C due to refusal of diabetic diet and medications (recommended regimen)

The patient has the right to refuse procedures.
At the same time, the providers have the discussion at the encounter and the patient still refuses.

Providers **should not have their quality percentages decrease** because of such thorough documentation of patient refusals post recommendation from clinical guidelines already documented in the encounters.

CMS should not transfer the burden of autonomy to providers by penalizing them in quality programs. CMS should ensure quality measures preserve **both patient rights and provider accountability**.

The screenshot below shows the decline for the ace inhibitor and the decline for the ARB.

- Medication declined (situation)
  - Administration of human anti-D immunoglobulin declined (situation)
  - Angiotensin converting enzyme inhibitor declined (situation)
  - Angiotensin II receptor antagonist declined (situation)
  - Antibiotic prophylaxis declined (situation)
  - Anticoagulation declined (situation)
  - Antihypertensive therapy declined (situation)
  - Antiviral therapy declined (situation)
  - Asthma medication declined (situation)
  - Beta blocker therapy declined (situation)
  - Bisphosphonates declined (situation)
  - Bupropion declined (situation)
  - Calcium channel blocker declined (situation)
  - Carvedilol therapy declined (situation)
  - Chemotherapy declined (situation)
  - Clopidogrel declined (situation)

Other sample screenshot which shows the decline for the statin as well others:

- Statin declined (situation)
- Strontium ranelate declined (situation)
- Teriparatide therapy declined (situation)
- Thrombolytic therapy declined (situation)
- Vitamin K prophylaxis declined (situation)
- Warfarin declined (situation)

- Medication dosage increase declined (situation)
- Medication review declined (situation)

- Obesity monitoring declined (situation)
- Osteoporosis monitoring declined (situation)
- Osteoporosis treatment declined (situation)

- Patient declined transportation (situation)
  - Ambulance transport to hospital declined (situation)

I can go on with the screenshots but you get the idea- there are SNOMED CT concepts for declining the procedures, medications, etc.

Providers and care delivery organizations should not be penalized with lower percentages and lower star ratings for patient refusals. Such documentation of patient refusals with the specific reasons as indicated in the above SNOMED CT examples and screenshots should show cause.

CMS should collaborate with NCQA, HL7, and SNOMED International to allow structured refusals to be excluded from screening and immunization quality denominators across Medicare Advantage, ACO, and MIPS programs.

This change would promote health equity, reduce provider burden, and more accurately reflect the patient-centered care CMS seeks to support.

The reason why I know this is because I worked in quality improvement in relation to managed care contracts for 8 years. I saw this firsthand.

Why This Is a Systemic Issue

- These measures:
  - Don't allow patient refusal as a valid denominator exclusion
  - Use “gap closure” logic enforced by payers, not clinicians
- Even documented informed refusal (e.g., “Patient refuses statin due to side effects”) does not protect the provider or plan from a quality penalty
- Patients remain in the denominator and count against compliance scores

Consequences

- Providers experience ethical conflict and pressure to override patient preference
- Plans, care delivery organizations, managed care contracts often run automated outreach, “gap” reports, and incentive programs to push providers harder
- Vulnerable populations (e.g., women, immigrant communities, people with prior trauma or vaccine hesitancy) may be over-targeted
- Providers are burned out, and patients lose trust

The **hospital transition-of-care measures** (e.g., those tracking follow-up visits, medication reconciliation, or outreach within 7–30 days of discharge) often **don’t account for patient refusal** — meaning:

- If a patient declines follow-up care, doesn’t answer calls, or refuses to schedule a visit,
- **The provider or hospital may still be penalized** for “failure to transition care appropriately,”
- Even when **every effort** was made on the clinician’s end.

CMS should formally recognize that patient empowerment includes the **right to refuse** preventive screenings and treatments, and that such refusals when properly documented should serve as **valid denominator exclusions** in quality measurement logic.

Colorectal cancer screening

https://ecqi.healthit.gov/ecqm/ec/2024/cms0130v12?qt-tabs_measure=measure-information

Breast cancer screening

https://ecqi.healthit.gov/ecqm/ec/2024/cms0125v12?qt-tabs_measure=measure-information

Diabetic Eye Exam:

https://ecqi.healthit.gov/ecqm/ec/2023/cms0131v11?qt-tabs_measure=measure-information

Hemoglobin A1C Frequency

CMS should reconcile payment policies with quality measure expectations. For example, A1C testing is often only billable once every 6 months, yet diabetes quality measures may implicitly expect more frequent testing to demonstrate control. Providers should not be penalized when payer rules prevent repeat testing. CMS should either align quality logic with billing frequency or allow documented exclusions for situations where reimbursement policy limits compliance.

https://www.cms.gov/medicare-coverage-database/view/lcd.aspx?lcdId=33431&ver=54

As real-time and continuous monitoring technology evolves, including the possibility of continuous A1C monitoring, CMS must modernize its quality measurement logic and payment systems to avoid penalizing providers for increased data granularity. Measures must evolve to interpret trends and context, not just discrete values. Payment systems should support continuous monitoring without restricting care or creating conflicting incentives.

CMS should begin preparing for a new generation of continuous biomarkers — including the potential for a continuous A1C monitoring device. While this innovation would revolutionize diabetes management, it risks being penalized under current quality logic. CMS should begin developing digital quality measure standards that accept **streamed, contextual biomarker data**, align with FHIR-based remote patient monitoring, and reward longitudinal improvement over single-point snapshots. This would enable innovation, reduce administrative burden, and ensure fair evaluation of high-tech, patient-centered care.

Incorporate Continuous Monitoring Data As New Quality Measures

- Continuous Glucose Monitoring
- Continuous BP monitoring
- Fitness Tracker monitoring

CMS should support development and adoption of quality measures that incorporate continuous glucose monitoring data, including metrics like Time in Range and hypoglycemia frequency, to

better reflect patient-centered diabetes management. CMS should also incorporate continuous or remote BP measurements that would better reflect blood pressure control, support hypertension management innovation, and align with increasing use of remote patient monitoring technologies. This would encourage adoption of advanced technologies and enable more precise quality assessment, beyond point-in-time lab values.

CMS should consider the full spectrum of wearable and remote monitoring technologies—including continuous glucose monitors, wearable ECGs, ambulatory blood pressure monitors, smart inhalers, and validated clinical activity trackers—in efforts to **modernize quality measurement and promote interoperability standards** that enable seamless data integration and use in value-based care.

Evolve Star Ratings with Remote Patient Monitoring Devices to Increase Patient Empowerment

CMS should **evolve Star Ratings** to incorporate data from remote patient monitoring devices and other digital health tools that reflect patient engagement and real-time health status. This approach would better recognize providers and plans supporting proactive, technology-enabled care and empower patients to take an active role in managing their health.

Member Attribution Logic versus Actual Provider Access to the Patient

CMS should allow providers and health plans to **flag beneficiaries for removal from attribution or quality measure denominators when multiple documented outreach attempts** (calls, letters, portal messages) are **unsuccessful**, or when **patients refuse care, relocate**, or experience access barriers. Structured FHIR-based reporting of **outreach** and **refusal data** should be incorporated into attribution logic and digital quality measure calculations to create fairness and transparency in value-based care models.

Providers should not be penalized in quality scores or star ratings for patients they cannot reach, especially when they are attributed retroactively and outreach efforts are clearly documented. A FHIR-based, structured outreach and contact status reporting mechanism should be introduced to enable **transparency** and **fairness** across value-based programs.

This would create parity between actual patient engagement capacity and quality accountability, and relieve provider burden in underserved or high-risk populations.

Many experienced physicians, including those deeply committed to quality care, have chosen early retirement or opted out of value-based contracts. A major factor has been the disconnect between rigid quality logic and the real-world dynamics of **patient behavior, access, and care refusal.**

CMS must modernize its attribution and quality measurement rules to recognize unsuccessful outreach, documented refusals, and access barriers as valid exclusions. This change would help retain experienced clinicians, reduce burnout, reduce burden, and preserve trust in value-based care models.

While CMS explores the role of patients, providers, technology vendors, and value-based care models, there is little clarity on how patient refusals of remote monitoring or preventive services should impact provider performance scores. This raises concerns about fairness in compliance and quality programs. I recommend CMS develop guidance that acknowledges patient choice in quality scoring systems.

The reason why I know this is because I worked in quality improvement and member attribution in relation to managed care contracts for 8 years. I saw this firsthand of the quality measures technical specifications not including exclusions for patient refusals. I remember mailing letters to the beneficiaries, being unable to leave voicemails and some of the beneficiaries would still be on the roster.

These concerns highlight a disconnect in the health IT ecosystem: providers may be penalized for patients' decisions, while technology vendors are not required to ensure full integration of remote monitoring data. CMS should address this imbalance to ensure the ecosystem supports patient-centered, value-based care without punishing clinicians unfairly.

Providers percentages decrease

https://www.uhcprovider.com/content/dam/provider/docs/public/reports/path/2025-PATH-Reference-Guide.pdf

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patient refusal is not acceptable exclusion

hence the reason why care delivery organizations, providers feel the pressure from the health plans to complete the colorectal screenings

Pneumococcal Vaccine